

FREE RESOURCE | THE MBA PA

APP Retention Risk Audit

A Systematic Framework to Pressure-Test Your Program's Retention Health

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The MBA PA

A practical diagnostic tool for Lead APPs, Program Directors, and CVOR Managers who want to identify hidden turnover risk across the five domains that quietly determine whether strong hires stabilize — or disengage long before anyone says it.

How to Use This Audit

- **For Lead APPs and program leaders:** Score your program honestly across five domains. This is a diagnostic, not a performance review — low scores are opportunities, not failures.
- **For institutional decision-makers:** Use the composite scores to build a business case for onboarding investment. Each domain maps to measurable turnover risk.
- **Re-audit quarterly:** Track changes over time. Improvement matters more than any single snapshot.

1) Role Clarity

Does your team know what "good" looks like — across every phase of their role?

Role ambiguity is the single most common driver of early APP turnover. When expectations are tribal knowledge — passed verbally from preceptor to trainee — every new hire starts with a different standard. Audit the following:

DIMENSION	WHAT TO AUDIT	RED FLAG IF...
Clinical scope	Written clinical privileges and procedure list per APP role	Scope defined by "what the surgeon prefers" rather than documented standards
Call expectations	Documented call frequency, response times, and backup structure	Call duties handed down verbally; no written rotation

Career ladder	Published advancement criteria (EVH independence, first-assist levels, precepting readiness)	No formal levels; advancement is "when I think you're ready"
OR expectations	Case-specific protocols for CABG, valve, LVAD, transplant	OR duties vary by surgeon preference with no written baseline
Non-OR duties	Clinic, rounds, database, teaching expectations clearly defined	Non-OR tasks accumulate without boundaries or acknowledgment

Scoring: Rate your program 1–5 on each dimension. 1 = undocumented / purely verbal. 5 = written, published, reviewed annually.

2) Onboarding Consistency

Is every new APP getting the same foundation — or does it depend on who trains them?

Onboarding inconsistency is the second-biggest predictor of differential team performance. When two APPs hired six months apart have wildly different competencies, the system — not the individuals — is the problem.

Onboarding Consistency Checklist

- **Written 90-day plan:** Does every new hire receive a structured onboarding document with week-by-week milestones? A verbal "you'll figure it out" is not a plan.
- **Preceptor assignment:** Is a specific preceptor named before day one, with protected time? Or is precepting ad hoc?
- **Skill gate criteria:** Are there explicit, written criteria for advancing from observing → assisting → independent for each procedure?
- **Check-in cadence:** Are there scheduled 30/60/90-day check-ins with the Lead APP — not just casual hallway conversations?
- **Remediation pathway:** If a trainee isn't meeting milestones, is there a documented process — or does it get handled informally until it's a crisis?
- **Completion standard:** Is there a formal sign-off process marking the end of onboarding? Without it, trainees stay in perpetual limbo.

Domains 3–5 & Composite Score

3) Feedback Loops

Are you catching issues before they become resignations?

The most dangerous retention signal is silence. APPs who stop giving feedback have already started disengaging. A healthy program has structured feedback flowing in both directions.

Feedback Loop Audit

- **Structured 1:1s:** Does every APP have a protected monthly or quarterly 1:1 with their Lead APP that isn't canceled when it gets busy?
- **Upward feedback:** Is there a mechanism for APPs to evaluate their preceptors and program leadership — anonymously if needed?
- **Exit interviews:** When someone leaves, is there a structured exit interview? If you've had recent departures without understanding why, that's a critical gap.
- **Stay interviews:** Are you talking to the APPs who are still there about what keeps them — or only focusing on those who leave?
- **Surgeon feedback:** Is there a structured process for surgeons to provide feedback on APP performance — not just complaints in the lounge?
- **Action on feedback:** When feedback is collected, does anything change? Nothing erodes trust faster than asking for input and ignoring it.

4) Team Integration

Does your culture retain — or quietly repel?

Culture is the retention variable most leaders acknowledge but few actively manage. Integration isn't about pizza parties — it's about whether APPs feel they belong in the room, are trusted with meaningful work, and have a voice in decisions that affect them.

SIGNAL	HEALTHY PROGRAM	WARNING SIGN	CRITICAL RISK
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OR trust	APPs first-assist across all case types independently	Surgeons request specific APPs; newer hires excluded	APPs treated as interchangeable techs rather than clinicians
Decision voice	APPs present at quality meetings; input solicited	APP input invited but consistently overridden without explanation	APPs not in the room for decisions that affect their work
Social cohesion	Team norms exist; new hires are actively included	Cliques form; new hires left to navigate alone	New hires eat lunch alone for the first 3 months
Professional growth	CE/conference support; role expansion pathways visible	Growth discussed but never funded or scheduled	No conversation about career progression has ever occurred

5) Work Design

Is the job itself sustainable — or is it designed for burnout?

Even the most engaged APP will eventually leave a job that's structurally unsustainable. Work design covers hours, call burden, scope creep, and whether compensation matches contribution.

Work Design Pressure Test

- **Hours reality check:** Are contracted hours close to actual hours? If everyone works 50+ hours on a 40-hour contract, that's a design problem — not individual endurance.
- **Call burden equity:** Is call distributed transparently and fairly — or do senior team members quietly offload onto newer hires?
- **Scope creep management:** When new responsibilities are added (database, teaching, QI projects), are old ones removed — or does scope only expand?
- **Compensation benchmarking:** When was the last formal market analysis of APP salaries in your region? "We pay competitively" without data is a retention vulnerability.
- **Schedule predictability:** Can APPs plan their lives more than two weeks out? Unpredictable schedules disproportionately drive experienced APPs away.
- **Recovery time:** Is there adequate time off between call shifts and regular shifts? Cumulative fatigue erodes clinical judgment and personal resilience simultaneously.

Composite Scoring & Action Planning

Domain Scoring Worksheet

Rate each domain 1–5 (1 = critical risk, 5 = fully systematized). Be honest — low scores are the most valuable data this audit produces.

1. Role Clarity

1

2

3

4

5

Score: ____

2. Onboarding Consistency

1

2

3

4

5

Score: ____

3. Feedback Loops

1

2

3

4

5

Score: ____

4. Team Integration

1

2

3

4

5

Score: ____

5. Work Design

1

2

3

4

5

Score: ____

Composite Score: ____ / 25

Interpreting Your Score

20–25	Systematized. Your program has structural retention health. Focus: maintain and refine.
15–19	Developing. Good instincts but gaps exist. Focus: shore up the lowest-scoring domain.
10–14	Vulnerable. Multiple domains need attention. Turnover risk is real and present.
Below 10	Critical. Retention is an immediate operational priority. Recommend structured intervention.

Action Planning: From Score to Strategy

Turn your audit results into a prioritization framework.

Priority Matrix

1. ● **Identify your lowest-scoring domain.** This is your highest-leverage intervention point. A one-point improvement in your weakest area reduces more turnover risk than polishing what's already working.
2. ● **Map each domain score to a concrete action.** "Improve feedback loops" is not an action. "Implement protected monthly 1:1s with documented agenda template by Q3" is an action.
3. ● **Assign ownership and timeline.** Each action needs one named owner and a deadline. Retention improvement without accountability is just good intentions.
4. ● **Re-audit in 90 days.** Measure the same five domains. Compare scores. Did your interventions move the needle? If not, the intervention — not the audit — needs adjustment.

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This is an operational leadership tool, not a clinical assessment or patient-care directive.

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